

LETTER OF INSTRUCTION — MEDICO-LEGAL REPORT

QIRC matter WC/2024/227 — Shepherd v Workers' Compensation Regulator

[Date] ____ August 2026

Dr Ravikumar Krishnaiah

Consultant Psychiatrist

Mind and Memory Service

[Practice address]

By email: [email]

Dear Dr Krishnaiah,

Re: Letter of instruction — medico-legal report on causation (WC/2024/227)

1. Who I am and what I ask you to do

I am the appellant in QIRC matter WC/2024/227, *Shepherd v Workers' Compensation Regulator* — an appeal under s 550 of the *Workers' Compensation and Rehabilitation Act 2003* (Qld) ("the Act") against the rejection of my claim for a psychological injury. The appeal is heard afresh (de novo) before Commissioner Dwyer. I am self-represented. At the material time I was employed by the Metro South Hospital and Health Service ("Metro South") as an AO3 Switchboard Officer at Logan Hospital. The onset of the injury is pleaded at 18 June 2024.

I ask you to prepare an independent medico-legal report for use in that appeal. I refer to it below as "the appeal report".

2. This report, and how it differs from the report for Metro South and from your report of 13 February 2025

Please treat this instruction as separate from two other documents, and please keep the scopes distinct:

(a) The report you are separately asked to prepare for Metro South concerns current capacity, functional restrictions and workplace adjustments. It is silent on the cause of the injury. **The appeal report is the reverse: its subject is causation.**

(b) Your report of 13 February 2025 was prepared for QSuper for income-protection purposes and addressed my *capacity* at that time. It was expressly marked as not for medico-legal use, and it did not address the statutory causation question in this appeal. **The appeal report is a different document answering a different question at a different point in time**, and I ask you to approach it as such (see paragraphs 6.7, 6.11 and 8).

3. The legal question, stated accurately

The question in this appeal is whether my employment was a *significant* contributing factor to the injury (s 32(1) of the Act). I set out the features of that test below only so that your report is directed to the correct legal standard, not to any particular answer:

Employment need only be a *significant* contributing factor. It need not be the only, nor the main, cause. (The word "major" is not the test.)

Other, non-employment contributors may coexist; they need not be outweighed by employment for the test to be met (*Adams v Q-COMP* [2015] ICQ 1 at [23], adopting *Hochen*).

An aggravation of a pre-existing condition is itself an injury where employment is a significant contributing factor to the aggravation (s 32(3)(b)), to the extent of the effects of the aggravation (s 32(4)).

I ask only that your report answer the question according to your own clinical judgment. **I do not ask you to reach any particular conclusion, and I would not wish you to.** A report that reflected instruction rather than your independent opinion would be of no use to me. Please answer each question below neutrally and on its clinical merits, including where the answer is adverse to my case.

4. The basis on which I ask you to write — assumed facts

This is the most important instruction in this letter. The facts of my employment — the role, the workload, the roster, the break, the complaints I made and the responses I received — are matters I must prove by my own evidence and the documents, not matters for you to establish. Accordingly:

I ask you to assume, for the purpose of your opinion, the facts set out in my signed statement dated [date] (Enclosure A), as corroborated by the chronology at Enclosure B and the admitted facts at Enclosure D.

Two directions follow, and they matter:

Please do not adopt, or treat as established, any conclusion, characterisation or finding of any other decision-maker. Where a document I provide contains such a conclusion, it is provided only for completeness of the record. The questions below are yours to answer independently, on your own clinical reasoning.

Where your own history from me, or your clinical view, differs from the assumed facts in any respect, **please say so expressly** rather than adopt or resolve the difference. Please do not seek out or rely on accounts from my colleagues or others about the disputed workplace events; if corroboration of those events is required, it will be given by the witnesses themselves.

The medical records at Enclosures E to I are provided for your clinical review — not as "assumed facts" about the workplace, but as the clinical record on which your diagnostic and causation opinions may draw.

5. Enclosures

Enclosure A — my signed statement dated [date].

Enclosure B — a chronology of the relevant events.

Enclosure C — the pleaded case, being the Amended Statement of Facts and Contentions (Amended Form 9A) dated 7 April 2026. This is provided for the **scope and context** of the matter — that is, to show you the matters in issue — and **not as facts you are asked to assume**. The case pleaded is **a course of management conduct** in relation to rostering, fatigue, pay and the handling of the concerns I raised. It is **not** a case of bullying or harassment, and I ask that the report describe the matter accordingly.

Enclosure D — an extract of the admitted facts from the Regulator's Form 24 Response (18 February 2026). The relevant admissions include: **the rostered break of 7 hours between my shifts of 17 and 18 March 2024** (the Respondent contending for an 8-hour minimum by a June 2020 agreement, against a 10-hour Award default); the criticality of the Switchboard function; and the admitted delay in the handling of the claim. These are matters the Respondent concedes.

Enclosure E — my general-practice records (Our Medical Ashmore), including the entries of 26 October 2022 (anxiety and ADHD) and any earlier stimulant (Vyvanse) use, and the entry of 16 November 2023.

Enclosure F — the Work Capacity Certificate of Dr Peter Hawes (which records first attendance for this injury on 1 July 2024, and is signed 8 September 2024).

Enclosure G — my certificate of capacity dated 3 July 2026 (certifying me fit for my substantive role with adjustments).

Enclosure H — your report of 13 February 2025.

Enclosure I — your own clinical records for me.

6. The questions

Please address the following, giving your reasons in each case (that is, "because A, B and C, in my opinion ..." rather than a bare conclusion):

6.1 Diagnosis, criteria and onset. Please state the diagnosis; the classificatory framework you apply (e.g. DSM-5, with the code); **the specific diagnostic criteria met, and when each was met**; and the date of onset as best the clinical record establishes it.

6.2 Differential diagnoses. Please identify the differential diagnoses you considered, and your reasons for including or excluding each.

6.3 Psychiatric background. Please address my background psychiatric history. In particular, please address **the entries of 26 October 2022 referring to anxiety and ADHD** (Enclosure E), and any earlier stimulant (Vyvanse) use, and state: whether there was any relevant pre-existing condition; and, if so, **whether the employment matters described in the enclosures aggravated any such condition and, if so, to what extent**. (I note the aggravation

provisions at paragraph 3 above.)

6.4 Sources. Please include in the report a **complete list of every document you reviewed and every person you saw**, with the dates and the duration of each examination. I ask for this specifically because the completeness of the source list is the first thing tested.

6.5 Causation (s 32(1)). On the assumed facts, please state whether my employment — and in particular the pleaded matters, including the rostering and fatigue sequence of 17 March – 1 May 2024 (the consecutive shifts, the admitted 7-hour break, my requests for fatigue relief, the refusal, and my consequent use of my own leave), the conditions of the Switchboard role as described in the assumed facts, and the handling of the concerns I raised — was **a significant contributing factor** to the injury of which onset is pleaded at 18 June 2024. Please give your clinical reasoning, and please distinguish between the **cause of onset** and any factors bearing only on the **subsequent course** of the condition.

6.6 The April–May 2024 material and the interval before presentation. The contemporaneous record shows that on 8 April 2024 I raised, in writing, the shift pairing and the minimum-break provision; that my manager escalated it to Human Resources on 9 April 2024; that I wrote again on 24 April 2024 recording more than two weeks without response; and that the request was refused on 1 May 2024. **What, if anything, do you make of the reports of fatigue recorded in April and May 2024, in the context of the diagnosis you made subsequently?** Separately, from a clinical perspective, what significance (if any) do you attach to the interval between the pleaded onset (18 June 2024) and the first medical presentation for this injury? (The Work Capacity Certificate at Enclosure F records first attendance for this injury on 1 July 2024; please take the presentation dates from the records themselves.)

6.7 Competing (non-employment) causes, placed in time. Please identify any non-employment factors you consider relevant to (a) the onset of the condition at or about 18 June 2024, and (b) its subsequent course. Your report of 13 February 2025 referred to "multiple life stressors, including relationship breakdown, job loss and bereavement". For each factor, please **state when in time it arose relative to 18 June 2024**, and address the significance, if any, of that timing for (a) the cause of onset and (b) the subsequent course. To assist with timing, please assume the following as facts: the cessation of my employment occurred in about October 2024 (and was later reversed by reinstatement); and the personal and relationship matters commenced from about December 2024. (I note the test does not require any competing cause to be outweighed: paragraph 3.)

6.8 Premorbid personality. Please address, **from your own clinical assessment**, whether any premorbid personality features bear on (a) the diagnosis and (b) the causation question at 6.5, and if so how — including whether such features are consistent with, or instead displace, the diagnosis you have made. I do not ask you to adopt or repeat any earlier characterisation; please reason from your own current assessment.

6.9 Mechanism. From a clinical perspective, please explain **whether, and if so how**, conditions of the kind described in the assumed facts — including sustained demand, the degree of control or autonomy, the adequacy of support, and fatigue arising from the rostering — can contribute to a condition of the kind you have diagnosed.

6.10 Prognosis and capacity. Please state your prognosis. As to capacity, please express your opinion on your own clinical judgment. In particular, please distinguish between (a) my capacity to perform the substantive role with reasonable adjustments, and (b) any current incapacity; and if there is any current incapacity, please state your opinion as to its cause — including whether it is attributable to the condition itself, to the consequences of my exclusion from the workplace since 3 July 2026, or to other factors. My certificate of capacity dated 3 July 2026 (Enclosure G) certifies me fit for my substantive role with adjustments; I provide it for your review and ask only that your opinion be your own. (The detailed assessment of adjustments is the subject of the separate Metro South report.)

6.11 Change in capacity over time. Your report of 13 February 2025 (Enclosure H) addressed my capacity at that time. From a clinical perspective, please explain how my capacity has evolved between that assessment and the present, and the reasons for any change.

7. Form of the report

Please prepare the report so that it can stand as expert evidence in the appeal. In particular:

Please do not include any "not for medico-legal use" limitation, or any similar footer or qualification. This report is prepared for use in QIRC proceeding WC/2024/227, and I ask you to confirm in writing before our consultation that it will not carry any such limitation.

Please include the **assumed-facts statement** in terms (see paragraph 4), and identify the assumptions on which your opinions depend.

Please reason from the assumed facts to your conclusions, rather than assert conclusions.

Please include the usual **expert's declaration** — that you understand your paramount duty is to assist the Commission and is not to the party who retains you, that the opinions expressed are your own, and that you have made all the inquiries you consider appropriate.

8. Oral evidence, testing, and fees

Please write on the basis that the report and your clinical file may be produced and tested. Under the QIRC guidance a medical report cannot be relied on without the expert also giving oral evidence and being available for cross-examination (Guide 7.6.5). Accordingly:

Please confirm your **availability to attend and give oral evidence**, and whether you are able to do so by **telephone or video**, which the Guide permits for expert witnesses.

I am responsible for your fees. Please provide a **fee estimate** for the report and, separately, for attendance to give evidence.

I am grateful for your assistance. If any part of this instruction is unclear, or if you require further material or a different form of the assumed facts before you can report, please tell me and I will provide it.

Yours sincerely,

Cory Lea Shepherd

Appellant (self-represented), WC/2024/227

[phone] · [email]

Enclosures: A – signed statement; B – chronology; C – Amended Form 9A (7 April 2026, scope/context only); D – Form 24 admitted-facts extract; E – GP records (Our Medical Ashmore); F – Hawes Work Capacity Certificate; G – certificate of capacity 3 July 2026; H – report of 13 February 2025; I – your clinical records.